

to all hospital staff. Health-care institutions should implement policies to address moral distress and emotional burden among their workers. In addition, medical education programmes should explicitly address abortion care and more fully integrate it into training. Successful models of these education programmes already exist and could be replicated or expanded both in the USA and globally.¹⁰ The Ryan (obstetrics and gynaecology) and RHEDI (family medicine) residency training programmes fully integrate abortion care into physician residency training and produce graduates who are more likely to provide abortion than those who don't attend these programmes. Finally, we must recognise that restricting abortion has far-reaching effects that expand into other aspects of health care and society, and that the consequences of the *Dobbs* decision are likely to persist for years to come.

We declare no competing interests.

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The evolving profile of eating disorders and their treatment in a changing and globalised world



It has been more than 70 years since the first eating disorder, anorexia nervosa, was included in the *Diagnostic and Statistical Manual of Mental Disorders* (DSM). Since then, recognition of a spectrum of disorders with eating difficulties has grown.¹ We draw on academic, clinical, and lived experience expertise to identify two

inter-related challenges in the understanding and treatment of eating disorders: the need for recognition of diverse presentations and increased attention to biological explanations in eating disorders.

Greater recognition is needed of the full spectrum of eating disorders and their diverse presentations globally.

Published Online
May 2, 2024
[https://doi.org/10.1016/S0140-6736\(24\)00874-2](https://doi.org/10.1016/S0140-6736(24)00874-2)

Feeding and eating disorders include anorexia nervosa, avoidant restrictive food intake disorder (ARFID), bulimia nervosa, binge eating disorder, pica, rumination disorder, and other specified and non-specified eating disorders.² Historically, the stereotype of a person with an eating disorder has been that of a thin, young, white female, often from a financially privileged background. Yet eating disorders may be even more common in people who are financially disadvantaged,^{3,4} are overweight or experience food insecurity,⁴ and in communities with multiple marginalised identities, including those relating to gender, ethnicity, and sexuality.³ Substantial efforts have been made to reflect this diversity and to listen to the voices of patients, families, activists, clinicians, and service providers within the eating disorders community.⁵

Body ideals vary depending on the country of residence and people's sociocultural status. But in the era of globalisation, mass media, and social media, western body size and physical fitness ideals have to some extent

infiltrated cultures in non-western and low-income and middle-income countries, including countries in the Middle East, Asia, and Oceania.^{6,7} Typical presentations of eating disorders, such as anorexia nervosa, have recently been reported even in African countries, including Ethiopia⁸ and Nigeria,⁹ where anorexia nervosa was thought to be almost non-existent.¹⁰ However, there is a large knowledge gap about the epidemiology of eating disorders across African nations.¹¹

Screening tools for eating disorders are biased to western cultures. For example, the Eating Attitudes Test-26¹² was originally developed with North American females, but a later evaluation of this measurement tool found that its factor structure across ethnic subgroups in Israel differed considerably both across the subgroups and from the typical three-factor structure found in Europe and North America.¹³ Indeed, there is considerable intercountry and intracontinental variation in eating behaviours, participation in research, discourse around mental health, and access to nutritious food, education, and health-care information.¹³ Accordingly, established barriers to research participation include limited access to or awareness of recruitment efforts,¹⁴ worries about misrepresentation, and distrust in medical research resulting from its history of discrimination.^{15,16} Strategies to improve recruitment from under-represented groups include setting clear recruitment targets from the start; promoting education and awareness about research projects;¹⁷ providing financial incentives to collect data from underserved communities;¹⁸ and engagement of people from marginalised populations and those with lived experience of eating disorders.

Similarly, treatment provision does not consistently encompass the diversity of eating disorders. Resources for treatment are often allocated to patients with anorexia nervosa because of the immediate medical risk, whereas people with more common eating disorders, such as binge eating disorder,¹⁹ are usually offered minimal clinical care after long waiting times. Although DSM-5 included ARFID, pica, and rumination disorder as new eating disorders,² this progress has not translated into investment in services to provide care for people with these disorders.

Encouragingly, the World Federation of Societies of Biological Psychiatry (WFSBP) developed new international guidelines for the pharmacological treatment of eating disorders in 2023, including the

Panel: Potential and established biological treatments for eating disorders

Neuromodulation²⁹

- Repetitive transcranial magnetic stimulation (anorexia and bulimia nervosa: small RCTs)
- Transcranial direct-current stimulation (bulimia nervosa: small single-session RCT)
- Deep brain stimulation (anorexia nervosa: open study and case reports)

Psychopharmacological treatments^{20,30–35}

- Antidepressants: fluoxetine (bulimia nervosa: RCTs; pica: case reports) and mirtazapine (ARFID: case reports)
- Antiepileptics: topiramate (bulimia nervosa: RCTs; binge eating disorder: RCTs)
- Antipsychotics: olanzapine (anorexia nervosa: RCTs) and levosulpiride (rumination disorder: open study)
- Attention deficit hyperactivity disorder medication: lisdexamfetamine (binge eating disorder: RCTs)
- Cannabinoid receptor agonist: dronabinol (anorexia nervosa: crossover RCT)
- Metabolic hormones: metreleptin (anorexia nervosa: case reports)
- Typical and atypical psychedelics: psilocybin (anorexia nervosa: feasibility study) and (es)ketamine (anorexia nervosa: case reports)
- Glucagon-like peptide 1 receptor agonists: liraglutide (binge eating disorder: pilot RCT), semaglutide (binge eating disorder: retrospective study), tirzepatide (no evidence in eating disorders), and retatrutide (no evidence in eating disorders)

Microbiome-based treatments^{36,37}

- Faecal microbiota transplant (anorexia nervosa: conflicting results of case reports)
- Prebiotics: compounds that foster growth or activity of beneficial microorganisms (no evidence)
- Probiotics: living microorganisms that are supposed to have health benefits (anorexia nervosa: feasibility RCT)

RCT=randomised controlled trial. ARFID=avoidant restrictive food intake disorder. The information in brackets refers to the highest level of evidence for the treatment.

novel diagnoses ARFID, pica, and rumination disorder. The guidelines had input from representatives from Africa, Asia, Australia, Europe, and North and South America, highlighting that global cooperation in this field is possible.²⁰ Such cooperation, however, requires representatives with a special interest and professional and lived experience in eating disorders, as well as knowledge of the needs of affected people, the cultural and ethnic contexts, and the political health strategies in the region that they represent.

An integrated, comprehensive, and internationally endorsed treatment approach should consist of different pathways for the diverse needs of individuals with eating disorders. One example of a pathway that considers neurodiversity is the Pathway for Eating disorders and Autism developed from Clinical Experience (PEACE).²¹ This is a therapeutic approach of adaptations for people with an eating disorder and traits of autism spectrum disorder that takes account of, for example, the sensitivity of people with traits of autism spectrum disorder in the domains of smell, touch, taste, sight, and sound to help support the complex needs of these patients.²¹ Other examples include the First Episode Rapid Early Intervention for Eating Disorders (FREED) care pathway for emerging adults (aged 16–25 years) with a recent onset of an eating disorder²² and the T1DE pathway for people with type 1 diabetes and disordered eating.²³

In addition to ensuring the diverse needs of people with eating disorders are met, there is a need for improved recognition and understanding of biological explanations of eating disorders among the scientific community and wider society. People with lived experience and their families have advocated for research into biological explanations. Notably, several cohorts that contributed to genetic studies of anorexia nervosa were funded by charities that included families with lived experience (eg, the Charlotte's Helix project, the Klarman Family Foundation, and the Price Foundation).²⁴ Such genetic research advanced the understanding of eating disorders as metabo-psychiatric disorders.²⁴ Research indicates that a higher genetic risk of anorexia nervosa is correlated with a lower genetic risk of a number of metabolic traits, some of which have a role in metabolic syndrome such as insulin resistance.²⁵ Additional evidence from genetic studies points to a differing underlying biology in anorexia nervosa compared with binge-type eating disorders.²⁶ However, as with other areas of research on

eating disorders, genetics research falls short in terms of its recruitment of a diverse range of participants. Thus far, all published genetic studies of anorexia nervosa have involved participants of European genetic ancestry, and this Eurocentric bias not only widens health inequalities but also constrains scientific discovery.²⁷ Encouragingly, recruitment efforts such as Born in Bradford and Genes & Health in the UK, and Depression Genetics in Africa (DepGenAfrica) are among the initiatives that are paving the way for widening participation of diverse populations in genetic studies.

However, the developing biological, genetic, and metabolic insights about eating disorders^{24–26} have not yet been translated into the way patients and carers are educated about eating disorders.²⁸ Furthermore, researchers have only taken the first steps to investigate biological treatments that are currently used for related mental health disorders, such as attention deficit hyperactivity disorder and depression, and for other metabolic disorders, including diabetes, obesity, or lipodystrophy, in people with eating disorders. Potential emerging and already established biological therapies include neuromodulation,²⁹ novel pharmacological treatments,^{20,30–35} and microbiome-based treatments (panel).^{36,37} However, the level of evidence varies for these treatments. Case reports have been published on the use of metreleptin³¹ and ketamine,³² and a feasibility study showed promising results for psilocybin therapy in anorexia nervosa.³³ Small randomised controlled trials (RCTs) provided preliminary evidence for repetitive transcranial magnetic stimulation and transcranial direct current stimulation in anorexia and bulimia nervosa²⁹ and for the glucagon-like peptide-1 receptor agonist liraglutide in binge eating disorder.³⁴ Several high-quality RCTs document the evidence for fluoxetine and topiramate in the treatment of bulimia nervosa, and for lisdexamfetamine and topiramate in the treatment of binge eating disorder.²⁰

Moving forward, clinical assessments and treatments must identify and address the differing needs of all individuals across the spectrum of feeding and eating disorders, including atypical presentations. Research and clinical approaches to eating disorders need to take account of different geographies, neurodiversity, and gender, racial, body, and cultural diversity. These approaches might involve considerations regarding clinical and dietary management in low-resource

settings and multidisciplinary training about eating disorders in local general hospitals.

Further, as knowledge of the biological aetiology of eating disorders develops, researchers should prioritise the inclusion of diverse participants and the translation of findings across the globe. Important considerations include the intersection between biological and cultural factors, including systems of social and systemic oppression. For example, a systematic review of genetic testing and counselling in low-income and middle-income countries established barriers associated with social stigma, religious and cultural beliefs, and deficiencies in service and educational provision as well as ethical and economic barriers.³⁸ Nevertheless, dissemination of biological education, for example via genetic counselling, has been welcomed by people with eating disorders.³⁹ It is essential to take a global approach and to consider cultural and social nuances in the response to eating disorders.

HH, JLK, and JT report funding from the UK Medical Research Council (MRC) for a double-blinded randomised controlled feasibility trial to test oral ketamine in people with anorexia nervosa and depression; the trial will start in 2025. HH and JT receive salary support from the National Institute for Health and Care Research (NIHR) Maudsley Biomedical Research Centre, part of the NIHR and hosted by South London and Maudsley NHS Foundation Trust in partnership with King's College London. HH is the principal investigator of a phase 2 clinical trial investigating the efficacy and safety of investigational COMP360 psilocybin treatment in anorexia nervosa. HLD and SAT declare no competing interests.

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A programme for greater health equity for the next UK Government



The UK is unhealthy, literally and metaphorically. Literally, since 2010, many health indicators have stopped improving, health inequalities are increasing between social groups and regions, and health for people who live in the most deprived areas is declining.^{1–4} Metaphorically, because the state of health of a population reflects how well that society is meeting population needs. The implication is that British society has stopped improving, inequalities are increasing, and social conditions for the poorest people are getting worse.

In our view policies to address this poor state of society and improve health equity should be a central topic for the UK election that will take place on July 4, 2024. If the first weeks of campaigning are anything to go by there seems to be general avoidance of this central topic, or dissembling about the substantial cuts to the social and economic fabric of society which have harmed health.^{1–4} Greater honesty is needed about what has driven the poor state of health in the UK and greater clarity is needed about strategies for positive action.

Statements about the economy illustrate our call for honesty. The current UK Prime Minister Rishi Sunak and the Chancellor of the Exchequer Jeremy Hunt, on the basis of gross domestic product (GDP) growth in the first quarter of 2024, said that the tough decisions they took have turned the economy around and that the UK now has the fastest growing economy in the G7.⁵ The non-partisan House of Commons Library published its own assessment of the GDP data.⁶ UK GDP in the first quarter of 2024 was 1.7% higher than pre-pandemic levels in the last quarter of 2019.⁶ That was the second slowest growth of the G7 countries; only Germany was slower.⁶ Predictions for 2024 by the International Monetary Fund and the Organisation for Economic Co-operation and Development (OECD)

again show the UK to have the slowest predicted growth of all the G7 countries, except Germany.⁶ Surely, to put things right, it is necessary to recognise what has gone wrong.

Much of what went wrong with respect to the social determinants of health equity in the period after 2010 comes under the rubric of austerity, imposed by a Conservative Party led coalition Government. In the 2020 Marmot Review,¹ we reported that in 2010 public sector expenditure had been 42% of GDP. Over the next decade, public sector expenditure went down year on year. By the end of the decade, public sector expenditure had become 35% of GDP.¹ An annual reduction of 7% is enormous. In 2023, total UK GDP was £2.687 trillion.⁷ 7% of that is £188 billion. At today's prices, annual public sector expenditure in 2019 was £188 billion less than it was in 2010. It is then not a surprise that relative child poverty went up—the steepest rise among 39 OECD countries;⁸ absolute measures of destitution increased; welfare payments apart from pensions did not keep pace with inflation; spending on education per pupil went down; the housing shortage became more marked and homelessness and rough sleeping increased; and increases in health-care expenditure fell sharply compared with historic trends.^{1,9} Alongside these major changes, came the slowest improvement in life expectancy in the UK during the decade after 2010, of any rich country except Iceland and the USA.¹

The cuts to public sector expenditure were regressive. In a 2024 Institute of Health Equity report, we plotted male and female life expectancy of each local authority in England in 2010–12, and showed that the shorter the life expectancy the steeper were the subsequent cuts in local authority spending power by 2019–20.³ It was as if, the greater the need for investment, the greater was the

Published Online
June 13, 2024
[https://doi.org/10.1016/S0140-6736\(24\)01243-1](https://doi.org/10.1016/S0140-6736(24)01243-1)